

RAAIDD Log

The main risks, actions, presumptions, problems, choices, and dependencies that will be tracked during the STADIOcare Data Science project are recorded in the RAAIDD log.

RAAIDD	Description
Risks	1. Inadequate or subpar historical data: STADIOcare's briefing pack notes inconsistent free-text fields, missing timestamps, and uneven coverage of historical data. This might make the model less reliable. 2. Data fragmentation: HR, appointments, admissions, and other operational data come from various systems and might not join regularly. 3. Modifications to operational procedures: Historical patterns may be impacted by modifications to hospital protocols, staffing guidelines, or data collection techniques. 4. Patient privacy and confidentiality: Healthcare data must be properly de-identified and governed because it may contain sensitive information.
Actions	Acquire the necessary historical datasets; perform data profiling; find missing values, duplicates, and inconsistencies; create trustworthy dataset joins; specify target variables; perform exploratory data analysis; create and compare predictive models; assess model performance; and record conclusions and suggestions for workforce and capacity planning.
Assumptions	The Group-wide data platform will give access to the necessary data sources; STADIOcare will supply adequate historical data; ADT data will contain trustworthy admission and discharge timestamps; pertinent datasets can be linked using suitable identifiers; and historical demand, occupancy, and length-of-stay patterns will contain predictive information.
Issues	Calculations for occupancy and length of stay may be impacted by missing or inconsistent timestamps. Comparisons may be impacted by variations in hospital definitions. The project scope or modeling strategy may need to be changed if there are significant gaps in historical data. Separate analytical techniques may be needed if datasets cannot be reliably joined.
Decisions	The project will concentrate on length-of-stay trends, bed utilization, and patient demand. Following data exploration, the best predictive

	methods will be chosen. Predetermined evaluation criteria will be used to compare models. Before modeling starts, STADIOcare stakeholders will decide on the final target-variable definitions.
Dependencies	Data request → data access → data-quality assessment → data preparation → exploratory analysis → model development → model evaluation → final recommendations. Each stage depends on the successful completion and outputs of the preceding stage.